

Health Equity Review: Orangeland

GECD Publishing · GECD Health Policy Studies
· 2025-06

Overview

This review examines health equity in Orangeland with a focus on women's access to care and the long-term care workforce. Figures in this review are drawn from national submissions and are not adjusted for differences in survey design. Cross-country comparisons should therefore be read as indicative rather than definitive.

The review covers the period 2020 to 2024 and draws on the GECD Health Statistics database, supplemented by national household surveys where comparable items exist.

Unmet care needs

Unmet medical need remains a useful summary indicator of access barriers. Across GECD members the share of women reporting unmet medical need averaged 6.8 per cent in 2024. Reported reasons differ markedly: cost dominates in some members while waiting time dominates in others.

Definitions are not fully harmonised. Some members ask about the past twelve months while others ask about the past four weeks, and the recall period alone can shift the estimate by several percentage points.

Long-term care workforce

Workforce density is a structural constraint on care quality. Orangeland reports 4.9 long-term care workers per 100 people aged 65 and over, below the GECD average of 6.3. Only four members report a lower density.

Turnover compounds the shortfall. Annual separation rates among long-term care

workers exceed 20 per cent in more than half
of reporting members .